

Focus Group Discussion Report

Perceptions of SRHR Services

Question 1: What do girls and young women in this community think about seeking SRHR services?

Discussion

Participants converged on SRHR being important in principle but underused in practice. Several described SRH services as necessary, confidential, and protective, framing access as essential for disease prevention and self-protection. At the same time, multiple participants admitted they personally only seek care when symptomatic rather than proactively, even while acknowledging that routine, preventive visits would likely be more beneficial since some conditions have no early symptoms.

A recurring point was that SRHR is widely misunderstood as being synonymous with sexual activity. One participant recounted having to clarify to a peer that seeking SRHR services doesn't imply someone is sexually active. The framework covers periods, contraception, pregnancy, and general reproductive health regardless of sexual history.

Negative provider interactions were also raised early: one participant described being misdiagnosed for a UTI as a sexually transmitted infection, and generally, participants felt providers reflexively assumed anything reproductive-health-related was sexual in origin.

Analysis

This suggests two distinct barriers operating simultaneously, a genuine knowledge/awareness gap (conflating SRHR with sex) and a practiced norm of reactive rather than preventive care-seeking, even among participants who articulate the value of prevention. This gap between stated belief and actual behavior is worth treating as its own theme rather than folding it into "stigma."

Question 2: What cultural barriers or community attitudes make it harder for girls to access these services?

Discussion

Several distinct mechanisms were raised:

FGM-practicing communities: complications from FGM (pain, bleeding, childbirth difficulty) were described as generating shame that discourages participants from seeking related care, compounding rather than only causing physical harm.

Family communication barriers: participants described being unable to discuss sexual/reproductive health with parents due to fear of judgment or being perceived as sexually active. Distrust of aunts and discomfort discussing periods or sex with fathers were both raised as normalized barriers.

Provider attitudes: multiple participants described feeling dismissed by gynecologists who characterized severe pain as "normal period pain," delaying diagnosis of more serious conditions for years in at least one case.

Religious/cultural framing: some participants noted that religious teachings discourage contraception specifically, creating tension between what clergy or community elders teach and what clinical guidance recommends.

Severe cases: one participant relayed a secondhand account (from a medical professional) of a patient from a community practicing genital sewing as FGM, and the complications that arose during childbirth, illustrating that "barrier" in some communities extends to serious physical harm, not just social discomfort.

One male participant noted that professionals' general absence from community spaces signals to boys/young men that these health topics aren't taken seriously, and that gender-segregated health education leaves boys without an equivalent peer support structure.

Analysis

Barriers span a spectrum from mild social discomfort (family conversations) to structural neglect (dismissive providers) to severe harm (FGM, forced closure practices). Grouping all of these under "cultural barriers" risks flattening very different severities and different points of intervention.

Family communication norms are addressable through education campaigns, while extreme practices require a different kind of intervention entirely.

Question 3: Where do girls get their SRH information and support, and is it trustworthy?

Discussion

Formal healthcare was not the dominant information source for most participants. Sources raised, in rough order of frequency:

AI tools (e.g., ChatGPT): one participant said she relies on AI more than she would confide in a sibling, but explicitly noted AI "can be manipulated" and isn't fully reliable, treating it as a supplementary rather than authoritative source.

Online communities and social media: described as valuable because they lower the barrier to learning compared to an in-person conversation, and because content on generational trauma/reproductive health was seen as actively shifting norms.

Peer networks: friends, including friends studying medicine, were cited as trusted sources.

Gynecologists: valued when trust was established, but participants noted that provider quality is inconsistent. While some dismiss concerns, others take them seriously.

Self-diagnosis/self-medication: several participants described routinely self-diagnosing based on online research or past experience, with explicit awareness of misdiagnosis risk (e.g., confusing a yeast infection with a UTI).

One participant disclosed she is personally developing an AI-based menstrual symptom triage tool, motivated by her own experience of providers dismissing her symptoms.

Analysis

The near-absence of formal healthcare as a first-line information source is a significant finding. It suggests distrust or inaccessibility of providers is pushing information-seeking toward informal, unverified channels (AI, peer networks, social media), which participants themselves flag as unreliable. This has direct implications for intervention design: peer- and tech-mediated channels may have more reach than clinic-based education campaigns, but need embedded accuracy safeguards.

Question 4: What happens socially, within families, or otherwise when it becomes known that a girl has sought SRHR services?

Discussion

Consequences were described as varying by the type of service sought:

- Contraception (especially visible methods like implants): participants described being labeled "immoral" and subject to gossip.
- HIV testing: described as carrying the highest level of stigma, with an automatic assumption of promiscuity.
- Counseling: discussed as less overtly stigmatized than the above two, though still not neutral.

A gendered double standard also emerged: participants noted that women can obtain monthly contraceptives from a chemist with a vague pretext (e.g., citing heavy periods) without much suspicion, while men purchasing condoms are also judged, despite condoms serving a protective function widely endorsed in the group's own discussion.

One participant noted this contradicted an initial assumption that only women face judgment around SRH purchases.

The "pads wrapped in newspaper" example recurred multiple times as shorthand for how menstruation-related purchases are treated as something to conceal even though menstruation is universal among women.

Analysis

The finding that stigma severity is service-specific (highest for HIV testing, lower for counseling) is a more precise and actionable finding than "stigma exists broadly": it suggests targeted destigmatization messaging could prioritize HIV testing and contraception specifically rather than treating all SRHR-seeking as equally stigmatized.

Question 5: Is the situation similar or different in other communities?

Discussion

Participants generally agreed the pattern (stigma, family silence, provider dismissiveness) is broadly shared across communities but is worse in specific contexts, pastoralist and some Muslim communities were named as facing compounded barriers, including practices like FGM and early marriage layered on top of the general stigma described earlier in the discussion.

There was also a note of cautious optimism: participants pointed to social media platforms as spaces where mothers and younger women are increasingly discussing generational trauma and reproductive health openly, which several felt was slowly shifting norms, even if change is uneven across communities.

Analysis

This suggests the core stigma dynamics identified in this FGD (family silence, provider dismissiveness, service-specific judgment) are likely generalizable beyond this specific participant group, with severity as the key variable rather than the presence/absence of the barriers themselves. This is useful for the study's generalizability claims, provided it's caveated as self-reported perception of "other communities" rather than direct data.

Cross-Cutting Observations for Analysis

- Behavior–belief gap: participants consistently endorse proactive/preventive SRHR use in the abstract but describe their own behavior as reactive/symptom-triggered, worth noting as distinct from stigma-driven avoidance.

- Privacy assumption reversal: one participant stated she would feel less safe in a one-on-one session with an unfamiliar provider than in a group setting, which cuts against a common assumption that privacy is the primary condition for disclosure comfort.
- Service-specific stigma gradient: HIV testing > contraception > counseling, in terms of stigma severity; a finding with direct implications for messaging design.
- Informal/digital channels as primary information sources: formal healthcare providers were consistently described as secondary or a last resort relative to AI tools, peer networks, and social media.
- Forward-looking participant input: the group itself proposed that future sessions shift from experience-sharing to solution-building, and specifically flagged the need to think beyond current participants (18+) toward younger girls, and male participant inclusion.